

MONROE COUNTY FIRE RESCUE — TRAUMA STAR

PRACTICE CHART — FICTITIOUS PATIENT — FOR QI TRAINING ONLY — NOT A MEDICAL RECORD

Practice Chart 1: IFT, Septic Shock, Ventilated + Sedated (target: 100%)

Date / OCA	2026-07-20 / PC-1001	Aircraft	Trauma Star North
Mission	Interfacility (Air)	Chief Complaint	Septic shock, ventilated
Pilot	A. Turner	Nurse / Medic	K. Lane, RN / J. Ford, Paramedic
Pickup / Dropoff	Mariners Hospital	Jackson South Medical Center	
Dispatch / Wheels Up	1402 / 1409	Land / Depart	1424 / 1441

Flight Narrative

Requesting agency: Mariners Hospital Emergency Department. Requesting provider Dr. Alan Reyes, MD, sending physician, arranged transfer; accepting physician Dr. Priya Kim, MD at Jackson South. Diagnosis: septic shock with acute respiratory failure. Sepsis is a time-sensitive emergency requiring prompt intervention. Sending facility lacks an ICU and continuous vasopressor management capability, not available at the pickup location; patient requires care that cannot be provided at the sending facility.

HPI: 64-year-old with two days of productive cough and fever, found hypotensive this morning. Events leading to transfer: progressive hypotension despite fluids. Prior to arrival the patient received 2L NS and was intubated by the sending facility. Infusions running prior to assessment: norepinephrine 8 mcg/min, propofol 30 mcg/kg/min. Labs: lactate 4.2, WBC 18.4, creatinine 2.1; blood cultures pending; CT chest pending, chest x-ray shows right lower lobe consolidation.

Rotor-wing air transport was chosen because the patient requires continuous critical care monitoring and titration beyond the scope of available ground providers. Ground transport time approximately 92 minutes per Google Maps versus 24 minutes by air; the time difference is clinically significant for this patient with escalating vasopressor requirements. Transported to Jackson South Medical Center for ICU and surgical capability that meets this patient's needs.

1410 BP 92/58 (MAP 68) HR 118 RR 18 SpO2 96% on FiO2 60%. Temp 101.2 F. Cabin temp 71 F. Glucose 142. 1420 BP 88/54 (MAP 62); norepinephrine titrated from 8 to 12 mcg/min at 1422 for MAP 62; reassessed 1430 MAP 71, hemodynamics stabilizing with ongoing vasopressor support. 1440 BP 96/60 (MAP 72) HR 110 SpO2 97%.

Ventilator (Hamilton): Ppeak 24, Pplat 19, Pmean 12, PEEP 8, AutoPEEP 0. VTe 420 mL (6 mL/kg IBW), ExpMinVol 7.6 L, fTotal 18, I:E 1:2, FiO2 60% weaned to 50% at 1428 for SpO2 97%. Cstat 38, Rinsp 11, RCexp 0.9. RASS -2 on propofol, target RASS -2 to -1, reassessed each vital set. Pain 2/10 at initial assessment using nonverbal scale, reassessed 2/10 at handoff.

Max altitude 1500 ft. Radio report given to Jackson South at 1435 with full patient report; sepsis alert activated by receiving facility. Patient remains at high risk of deterioration without continuous critical care monitoring and titration. Care transferred to ICU team on arrival.

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